

MEMORANDUM

AUTHOR: Mohamed Noor Hussein **DATE:** June 26, 2026 **SUBJECT:** Transferability of Minnesota's Medicaid Enforcement-Record Indicators to Washington State Adult Family Homes and HCBS

Executive Summary

This memorandum analyzes the program-integrity evidence base compiled from Minnesota's 2024–2026 Medicaid fraud enforcement wave to identify review-priority indicators transferable to Washington State's oversight of Adult Family Homes (AFHs) and Home and Community-Based Services (HCBS). Minnesota's enforcement wave — which culminated in a \$2 billion federal funding threat and one of the largest Medicaid fraud takedowns in U.S. history — followed a period in which the state's own auditors described proactive, data-driven administrative oversight at the licensing and enrollment stages as underdeveloped.

The structural features of Minnesota's Housing Stabilization Services (HSS), Integrated Community Supports (ICS), and Individualized Home Supports (IHS) programs are analogous to the operational models of Washington's AFHs and HCBS waivers. Adapting the review-priority indicator typology developed from the Minnesota evidence base gives Washington State a public-data foundation for a review-priority layer; it is a starting point for further oversight work, not a replacement for claims-informed review.

1. Structural Analogies: Minnesota to Washington

Minnesota's exploited programs share critical characteristics with Washington's AFHs and HCBS: * **Decentralized, residential service delivery:** Services are delivered in small residential settings or individual homes, lacking the centralized administrative infrastructure and peer oversight of traditional institutional care (e.g., nursing facilities). * **Direct-care staffing models:** Heavy reliance on unlicensed or minimally credentialed direct-care workers (e.g., PCAs, behavioral technicians) performing high-volume, hourly services. * **Low barriers to entry:** Rapid expansion of new provider entities with minimal capitalization requirements. * **Vulnerable beneficiary populations:** Serving individuals with severe mental illness, cognitive disabilities, or substance use disorders who are often unable to independently verify or complain about the services billed on their behalf.

The Minnesota Office of the Legislative Auditor (OLA) explicitly warned in 2017 [1] that the state's HCBS oversight relied almost entirely on reactive complaint investigations rather than proactive financial audits, and that licensing requirements did not collect adequate financial documentation. This exact gap allowed the 2021–2025 fraud wave to occur.

2. Most Transferable Review-Priority Indicators

Based on the empirical evidence from Minnesota's charging documents and audits, the following indicators are the most predictive and transferable to Washington:

A. Administrative and Enrollment Anomalies

- **Explosive Program/Provider Growth (RF-01, RF-02):** In Minnesota, EIDBI spending grew from \$600,000 to over \$400 million, and HSS grew from \$21 million to \$104 million in three years [2][3]. Washington must establish baseline growth expectations for AFHs/HCBS and trigger automatic audits when aggregate spending or individual provider billing accelerates beyond beneficiary population growth.
- **Phantom or Residential Business Addresses (RF-09):** The largest Minnesota case (\$11M) operated for years out of a mailbox [4]. Washington licensing must verify physical, commercial operating locations before enrollment and during revalidation.
- **Opaque or Concealed Ownership (RF-07):** Fraudulent providers frequently hid their ownership of residential properties (a violation of Medicaid rules) to control beneficiary housing and capture their billing data [2]. Washington must rigorously enforce 42 CFR § 455.104 ownership disclosures and cross-reference property records against provider registries.

B. Billing and Claims Data Anomalies

- **Impossible Hours and Overlapping Locations (RF-10, RF-12):** Minnesota providers repeatedly billed for more than 24 hours in a day, billed while the provider or recipient was out of the country, or billed for simultaneous services at different locations [5]. Washington's MMIS (Medicaid Management Information System) must have hard-coded edits to reject impossible concurrent billing and mandate Electronic Visit Verification (EVV) with geolocation.
- **Billing for Deceased Recipients (RF-13):** Multiple Minnesota providers billed for services days or weeks after a recipient died [2][5]. This indicates a failure to cross-check claims data against state vital records. Washington must implement automated, daily death-record cross-matches prior to payment.

- **Identical, Cloned Documentation (RF-14):** County investigators in Minnesota flagged the state's largest HSS biller for submitting identical 15-minute service notes for every client, every day [6]. Washington should utilize AI or text-analytics to scan HCBS service notes for cloning or boilerplate language.

C. Beneficiary Recruitment and Kickbacks

- **Fraudulent Inducements (RF-13):** Minnesota providers paid cash kickbacks to parents of autistic children and offered cash to homeless individuals outside shelters to obtain their Medicaid numbers [3][6]. Washington must train case managers and social workers to ask beneficiaries how they selected their AFH or HCBS provider and whether they were offered any financial inducement.

3. The Cost of Reactive Oversight

Minnesota's experience demonstrates that waiting for law enforcement is too late. By the time the DOJ and MFCU brought charges in 2024–2026, hundreds of millions of dollars had been lost, programs had to be shuttered (HSS), and vulnerable individuals were harmed (including a deceased ICS recipient whose provider continued to bill) [2].

Furthermore, Minnesota's oversight agencies frequently ignored early warnings from county-level managed care organizations and frontline workers. In one documented instance, a county fraud investigator explicitly warned the state that they were "flying blind" on HSS fraud a full year before the state took administrative action [6].

4. Recommendations for Washington State

1. **Implement Proactive Financial Audits:** Do not rely solely on health-and-safety licensing inspections. Adopt the 2017 MN OLA recommendation to require routine financial documentation and conduct random financial audits of AFHs and HCBS providers.
2. **Deploy Predictive Data Analytics:** Utilize claims data to identify billing outliers (e.g., providers billing the maximum allowable units for every client) before payment is disbursed, rather than in "pay and chase" recovery efforts.
3. **Establish a Centralized Fraud Tip Triage:** Ensure that warnings from MCOs, county social workers, and beneficiaries are reviewed through a documented triage process and, where legally supported, referred for appropriate claims review or program-integrity action under existing agency standards.
4. **Mandate Strict Revalidation:** Follow Minnesota's eventual corrective action by requiring unannounced site visits and rigorous background studies during the provider revalidation process.

References

[1] Minnesota Office of the Legislative Auditor. "Home- and Community-Based Services: Financial Oversight." February 2017. <https://www.auditor.leg.state.mn.us/ped/2017/hcbssum.htm> [2] U.S. Department of Justice. "Minnesota Health Care Fraud Takedown Results in Charges Against 15 Defendants in Over \$90M in Fraud." May 21, 2026. [3] IRS Criminal Investigation. "Six Additional Defendants Charged, One Defendant Pleads Guilty in Ongoing Fraud Schemes." December 18, 2025. [4] Minnesota Attorney General's Office. "Attorney General Ellison charges five with stealing more than \$10M in two Medicaid fraud prosecutions." June 19, 2024. [5] Minnesota Attorney General's Office. "Attorney General Ellison announces Medicaid fraud charges against seven as part of National Health Care Fraud Takedown Day." June 23, 2026. [6] KARE 11. "Flying blind: State agency missed or ignored repeated housing fraud warnings." September 11, 2025.